

FLUCOR NIGHT SOFT GELATIN CAPSULE

WIFLUxx-0 (MY)

COMPOSITION

Each softgel contains:	
Paracetamol	250.0 mg
Pseudoephedrine HCl	30.0 mg
Chlorpheniramine maleate	2.0 mg

DESCRIPTION

Oblong, dark green soft gelatin capsule printed with 'Flucor', filled with transparent green liquid.

ACTIONS AND PHARMACOLOGY

Paracetamol, as an analgesic, may act predominantly by inhibiting prostaglandin synthesis in the central nervous system (CNS) and, to a lesser extent, through a peripheral action by blocking pain-impulse generation. The peripheral action may also be due to inhibition of prostaglandin synthesis or to inhibition of the synthesis or actions of other substances that sensitize pain receptors to mechanical or chemical stimulation. Paracetamol probably produces antipyresis by acting centrally on the hypothalamic heat-regulating center to produce peripheral vasodilation resulting in increased blood flow through the skin, sweating, and heat loss. The central action probably involves inhibition of prostaglandin synthesis in the hypothalamus.

Pseudoephedrine acts on alpha-adrenergic receptors in the mucosa of the respiratory tract, producing vasoconstriction. The medication shrinks swollen nasal mucous membranes; reduces hyperemia, edema, and nasal congestion; and increases nasal airway patency. Also drainage of sinus secretions may be increased and obstructed Eustachian ostia may be opened.

Chlorpheniramine maleate is a sedating antihistamine that causes a moderate degree of sedation. Its anticholinergic actions provide a drying effect on the nasal mucosa.

PHARMACOKINETICS

Paracetamol is readily absorbed from the gastrointestinal tract with peak plasma concentrations occurring about 10 to 60 minutes after oral administration and is distributed into most body tissues. The elimination half-life of paracetamol varies from about 1 to 3 hours. It is metabolized predominantly in the liver and excreted in the urine mainly as the glucuronide and sulphate conjugates, less than 5% is excreted as unchanged paracetamol.

Pseudoephedrine is readily absorbed from the gastrointestinal tract. It is resistant to metabolism by monoamine oxidase and is largely excreted unchanged in the urine together with small amounts of its hepatic metabolite. It has a half-life of about 5 to 8 hours; elimination is enhanced and half-life accordingly shorter in acid urine. Small amounts are distributed into breast milk.

Chlorpheniramine maleate is absorbed relatively slowly from the gastrointestinal tract, peak plasma concentrations occurring about 2.5 to 6 hours after oral administration. It appears to undergo considerable first-pass metabolism. About 70% of chlorpheniramine in the circulation is bound to plasma proteins. Values ranging from 2 to 43 hours have been reported for the half-life. Chlorpheniramine maleate is widely distributed in the body and is extensively metabolised. Metabolites include desmethyl- and didesmethyl- chlorpheniramine. Unchanged drug and metabolites are excreted primarily in the urine; trace amounts found in the faeces.

INDICATIONS

For the temporary relief of running nose and sneezing, minor aches and pains, headache, fever and nasal congestion associated with the common cold.

CONTRAINDICATIONS

- In patients with a known hypersensitivity to Paracetamol, Pseudoephedrine or Chlorpheniramine.
- In patients under treatment with MAO inhibitor and within 2 weeks of stopping such treatment.
- In patients with severe hypertension or severe coronary heart disease.

WARNING

This preparation contains PARACETAMOL. Do not take any other paracetamol containing medicines at the same time. This product contains Sodium Metabisulphite that may cause serious allergic type reactions in certain susceptible patients. Do not use if know to be hypersensitive to bisulphites. Allergy Alert: Paracetamol may cause severe skin reactions. Symptoms may include skin reddening, blisters or rash. These could be signs of a serious condition. If these reactions occur, stop use and seek medical assistance right away.

PRECAUTIONS

- Paracetamol should be given with care to patients with impaired kidney or liver function and to patients with alcohol dependence.
- Pseudoephedrine is distributed into breast milk; hence use by nursing mothers is not recommended as infants, are at higher risk of side effects from sympathomimetic amines. It should be used with caution in infants, especially newborn and premature infants because of higher than usual risk of side/adverse effects. Use of pseudoephedrine in geriatrics may require adjustment of dosage as elderly patients are more likely to have age-related prostatic hypertrophy.

- Chlorpheniramine maleate will cause drowsiness. Those affected should not drive or operate machinery; alcohol should be avoided. It should be used with care in conditions such as glaucoma, urinary retention and prostatic hyperplasia.
- Do not exceed recommended dosage. Consult general practitioner should symptoms persist.
- Patients with hearts disease, thyroid disease, high blood pressure, breathing problems, persistent or chronic cough, asthma, diabetes, chronic bronchitis, glaucoma or difficulty in urination due to enlargement of the prostate gland should consult a doctor before using this product.

MAIN SIDE/ADVERSE EFFECTS

- Haematological reactions including thrombocytopenia, leukopenia, pancytopenia, neutropenia and agranulocytosis have been reported as side effects of paracetamol with skin rashes and other hypersensitivity reactions occur occasionally.
- The commonest adverse effects of pseudoephedrine include tachycardia, anxiety, restlessness and insomnia; skin rashes and urinary retention have occasionally occurred. Hallucinations have been reported rarely.
- Adverse effects with chlorpheniramine appear to be less frequent or rare and may include difficult or painful urination, drowsiness, increased sweating, loss of appetite and paradoxical reaction.
- Cutaneous hypersensitivity reactions including skin rashes, angioedema, Stevens Johnson Syndrome/Toxic Epidermal Necrolysis have been reported.

PREGNANCY

Problems in humans have not been documented.

LACTATION

Pseudoephedrine and small amount of antihistamine (chlorpheniramine) are distributed into breast milk. Hence, use by nursing mothers is not recommended because of the higher and usual risk to infants, especially newborn and premature infants, of side effects from sympathomimetic amines (pseudoephedrine) and unusual excitement or irritability in infants (chlorpheniramine).

DRUG INTERACTIONS

Concomittant use this product with other sympathomimetic agents such as decongestants, tricyclic antidepressants, appetite suppressants and amphetamine - like psychostimulants or with MAO Inhibitor, which interfere with the catabolism of sympathomimetic amines, may occasionally cause a rise in blood pressure. Because of its Pseudoephedrine content, this product may partially reverse the hypotensive action of drugs which interfere with sympathetic activity including bretylium, bethadine, guanethidine, deprisoquine, methyldopa, alpha and beta adrenergic blocking agents.

OVERDOSE

Symptoms: Toxic symptoms include vomiting, abdominal pain, hypotension and sweating. The most serious adverse effect of acute overdose of Paracetamol is a dose-dependent, potentially fatal hepatic necrosis. In adults, hepatotoxicity may occur after ingestion of a single dose of 10 to 15g of Paracetamol, a dose of 25g or more is potentially fatal. Symptoms during the first 2 days of acute poisoning by Paracetamol do not reflect the potential seriousness of the intoxication. Major manifestations of liver failure such as jaundice, hypoglycemia and metabolic acidosis may take at least 3 days to develop.

Treatment: In cases of overdose, methods of reducing the absorption of ingested drug are important. Gastric lavage is essential even if several hours have elapsed. Prompt administration of 50g activated charcoal and 500mL iced mannitol 20% by mouth may reduce absorption. If the history suggests that 15g Paracetamol or more has been ingested, administer one of the following antidotes:
i) Acetylcysteind 20% IV
ii) Oral methionine
iii) Oral acetylcysteine 5%

DOSAGE AND ADMINISTRATION

Adults and children 12 years of age and over:
Oral use, 2 softgels every 4 hours; not exceeding 8 softgels in 24 hours.

Note: The information given here is limited. For further information, consult your doctor or pharmacist.

Storage: Store below 25°C.

Presentation/Packing:
Blister pack of 8's, 10 x 10's

Product Registration Holder / Manufactured by: HOVID Bhd.
121, Jalan Tunku Abdul Rahman (Jalan Kuala Kangsar),
30010 Ipoh, Perak, Malaysia.

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